

EVIDENCE LOG

For Sleep Apnea VA Disability Claims

The Smoking Gun Evidence

After years of navigating the VA claims process—as veterans ourselves—we discovered something that changes everything: **the single most powerful piece of evidence you can submit isn't officially part of a Fully Developed Claim.**

"No more convincing. Just corroboration."

When you walk into a C&P exam without this documentation, the examiner's job is to **investigate whether your condition exists and how severe it is**. They're playing detective. You're trying to convince them.

When you walk in WITH a properly completed Evidence Log, the dynamic completely shifts. The examiner's job becomes **corroborating what you've already documented**. They're not investigating—they're verifying. That's a fundamentally different exam.

How VA Raters Actually Evaluate Your Claim

Every VA disability rating—regardless of condition—is based on three core factors. Raters are trained to look for evidence of these specific elements:

Factor	What It Means	What Raters Look For
FREQUENCY	How often do your symptoms occur?	Daily? Weekly? Constant? Episodic? Document specific patterns.

Factor	What It Means	What Raters Look For
SEVERITY	How intense are your symptoms when they occur?	Mild, moderate, severe? Specific descriptions of pain levels, intensity of panic attacks, etc.
FUNCTIONAL IMPACT	How do symptoms affect your daily life and ability to work?	Can't work? Can't sleep? Can't maintain relationships? Specific limitations.

Why This Evidence Log Works: The template you're about to use is specifically designed to capture all three factors in a format that's easy for raters to process. When a rater opens your file and sees 30 days of documented symptoms with frequency, severity, and functional impact clearly recorded, they don't have to guess or interpret. The evidence speaks for itself.

DO NOT FILE ANY CLAIM UNTIL THIS LOG IS COMPLETED AND INCLUDED WITH YOUR SUBMISSION

Sleep Apnea: Condition-Specific Guidance

Sleep Apnea is rated under 38 CFR § 4.97, Diagnostic Code 6847. Unlike many conditions, sleep apnea ratings are heavily dependent on **objective medical evidence** (sleep studies) combined with documentation of **required treatment and residual symptoms**.

What C&P Examiners Focus On:

- **Sleep study results:** AHI (Apnea-Hypopnea Index) score, oxygen desaturation levels
- **Treatment compliance:** CPAP/BiPAP usage, hours per night, nights per week
- **Persistent daytime hypersomnolence:** Excessive daytime sleepiness DESPITE treatment
- **Respiratory failure:** Need for supplemental oxygen, CO2 retention
- **Functional impact:** Falling asleep at inappropriate times, cognitive impairment, safety concerns

Sleep Apnea Rating Levels:

Rating	Criteria
0%	Asymptomatic but with documented sleep disorder breathing
30%	Persistent daytime hypersomnolence
50%	Requires use of breathing assistance device such as CPAP machine
100%	Chronic respiratory failure with carbon dioxide retention, cor pulmonale, OR requires tracheostomy

CRITICAL FOR 50% RATING: THE VA REQUIRES PROOF THAT YOU USE THE CPAP, NOT JUST THAT YOU WERE PRESCRIBED ONE. YOUR EVIDENCE LOG COMBINED WITH CPAP MACHINE DATA DOWNLOADS PROVIDES THIS PROOF.

How to Use the Evidence Log for Sleep Apnea:

Your log should document both your CPAP compliance AND your residual symptoms. Many veterans get stuck at 50% when they could qualify for consideration of additional ratings based on secondary

conditions (like depression from chronic fatigue) or demonstrating that symptoms persist despite treatment.

Strong Entry Example: "CPAP usage: 6.5 hours (mask seal issues, woke twice to adjust). Still exhausted upon waking despite full night with CPAP. Fell asleep at desk at work around 1400—coworker had to wake me. Near-miss driving home—caught myself drifting in lane. Wife says I still stop breathing even with CPAP on (she counted 3 episodes). Headache most of morning. Had to cancel evening plans due to fatigue."

The strong entry documents compliance (6.5 hours), persistent symptoms despite treatment (exhausted, fell asleep at desk, near-miss driving), and functional impact (couldn't complete work tasks alertly, had to cancel plans). This demonstrates that the 50% rating criteria may not fully capture your impairment.

Weak Entry Example: "Used CPAP. Still tired."

Completed Example: 7-Day Evidence Log

Below is an example of a properly completed Evidence Log. Notice how each entry captures specific details about symptoms, their severity, and their impact on daily functioning. Your log should follow this format.

Date	CPAP Data	Symptoms Despite Treatment	Functional Impact
Mon 1/6	5.2 hrs AHI: 3.2 2 mask leaks	Woke exhausted despite CPAP. Headache until 1000. Wife says I stopped breathing twice that she noticed.	Fell asleep in morning meeting—very embarrassing. Energy crash at 1400. Needed coffee to drive home safely.
Tue 1/7	6.8 hrs AHI: 1.8 Good seal	Better night but still tired by afternoon. Excessive yawning starting at 1300.	Brain fog slowed me down—estimate 60% of normal. Took 20 min "rest" in car at lunch.
Wed 1/8	4.1 hrs AHI: 4.5 Removed at 0300	Woke gasping, removed mask in frustration. Severe fatigue all day. Morning headache 8/10.	Called in sick. Napped twice for 1-2 hrs during day. Still felt unrested. Cancelled evening gym session.
Thu 1/9	7.0 hrs AHI: 2.1 Good seal	Good compliance night. Still daytime sleepiness 6/10. Near-sleep episodes driving home.	Wife had to take over driving. Fell asleep on couch at 1830. Missed family dinner.
Fri 1/10	5.5 hrs AHI: 2.8 1 mask leak	Woke with dry mouth despite humidifier. Fatigue 7/10 by afternoon. Brain fog (forgot coworker's name).	Made error at work—had to re-do 2 hrs of work. Too tired for hobbies.
Sat 1/11	6.2 hrs AHI: 2.4	Weekend but still need CPAP. Tired despite sleeping in. Afternoon nap 2.5 hrs.	Could not help with yard work as planned. Wife frustrated. Fell asleep during movie.

Date	CPAP Data	Symptoms Despite Treatment	Functional Impact
Sun 1/12	7.5 hrs AHI: 1.5	Best night this week. Still tired by 1400. Needed rest before dinner.	Low energy for family activities. Kids asked why I'm "always tired." Napped 1.5 hrs.

Your Evidence Log: 30-Day Template

Instructions: Complete one row per day for at least 30 consecutive days before your C&P exam or claim submission. Be specific, honest, and focus on frequency, severity, and functional impact. Print additional copies as needed.

VETERAN NAME: _____

CONDITION: Sleep Apnea

CLAIM #: _____

START DATE: _____

Date	CPAP Data (HRS/AHI)	Symptoms Despite Treatment (fatigue, sleepiness, headaches, etc.)	Functional Impact (work, driving, daily activities)
Day 1 __/__/__			
Day 2 __/__/__			
Day 3 __/__/__			
Day 4 __/__/__			
Day 5 __/__/__			
Day 6 __/__/__			

Date	CPAP Data (HRS/AHI)	Symptoms Despite Treatment (fatigue, sleepiness, headaches, etc.)	Functional Impact (work, driving, daily activities)
Day 7 __/__/__			

Date	CPAP Data	Symptoms Despite Treatment	Functional Impact
Day 8 _/_			
Day 9 _/_			
Day 10 _/_			
Day 11 _/_			
Day 12 _/_			
Day 13 _/_			
Day 14 _/_			

Date	CPAP Data	Symptoms Despite Treatment	Functional Impact
Day 15 _/_			
Day 16 _/_			
Day 17 _/_			
Day 18 _/_			
Day 19 _/_			
Day 20 _/_			
Day 21 _/_			

Date	CPAP Data	Symptoms Despite Treatment	Functional Impact
Day 22 _/_			
Day 23 _/_			
Day 24 _/_			
Day 25 _/_			
Day 26 _/_			
Day 27 _/_			
Day 28 _/_			

Date	CPAP Data	Symptoms Despite Treatment	Functional Impact
Day 29 __/__/__			
Day 30 __/__/__			

VETERAN CERTIFICATION: I certify that the information recorded in this Evidence Log is true and accurate to the best of my knowledge. I understand that false statements may result in denial of benefits or other penalties.

Veteran Signature

Date

Witness (optional)

Date

DISCLAIMER: This template is provided for educational purposes only. It is not legal advice and does not constitute representation before the VA. Consult with an accredited Veterans Service Organization (VSO) or VA-accredited attorney for assistance with your specific claim.